

Dietitian Consultation Note

GI Associates of Boston | Dr. Elena Hartley, MD

Patient: Jane Doe | DOB: 1988-03-14 | MRN: 00142837

Visit date: 2026-05-28 RD: Melissa Grant, RD, LD Referral from: Dr. Elena Hartley

REASON FOR REFERRAL

Active UC flare with iron deficiency anemia, low vitamin D, weight loss risk. Nutritional optimization during steroid taper.

DIETARY ASSESSMENT

Current diet: Self-described as gluten-free due to symptom sensitivity. Avoiding raw vegetables, seeds, nuts, dairy. Predominantly rice, chicken, bananas, white bread.

24-hr recall (estimated):

Calories: ~1,400 kcal (goal 1,900-2,100)

Protein: ~55g (goal 80-90g)

Iron: ~6mg (goal 18mg for premenopausal women, 27mg if deficient)

Vitamin D: ~150 IU (goal 2,000 IU with supplementation)

Fiber: ~8g (low-fiber diet appropriate during active flare)

RECOMMENDATIONS

- PROTEIN:** Add soft-cooked eggs, canned fish (salmon/tuna), smooth nut butters (tolerated), Greek yogurt if dairy tolerated -target 25-30g per meal.
- IRON:** Cook in cast iron when possible. Pair iron-rich foods with vitamin C sources (orange juice, kiwi, bell peppers). Avoid tea/coffee within 1 hour of iron-rich meals. Iron supplement (ferrous sulfate 325mg) as prescribed by Dr. Elena Hartley.
- VITAMIN D:** 2000 IU supplement daily with largest meal. Salmon 2x/week if tolerated.
- CALORIES:** Add calorie-dense, low-residue snacks: avocado, olive oil drizzle on rice, nut butter on white toast, protein shakes (Ensure or Boost) between meals.
- AVOID DURING FLARE:** raw brassicas, legumes, high-fructose corn syrup, alcohol, carbonated beverages, excess caffeine.
- REINTRODUCTION PLAN:** As flare resolves, gradually reintroduce cooked vegetables starting with zucchini, carrots, and peeled sweet potato.

FOLLOW-UP

RTC in 4 weeks. Food diary requested. Monitor weight weekly.